

Rehab Evaluation Audit

Patient: askew, James

MRN: 6744

Facility: PH OPS OF DALLAS

Discipline: PT

Evaluator: Maurice Chua

Evaluation DOS: 5/13/2026

Completed: 5/13/2026 12:01:49 PM EDT

Certification Period:
5/13/2026 – 6/11/2026

Payer: Medicare Part A

Overall Score

26 / 31 (83.9%)

Pass: 7 · Partial: 3 · Fail: 0 · Unable to Validate: 1

External Validation Pending: 1 criterion / 3 points (excluded from the score above).

Audit Criteria

Section	Criterion	Score	Status	Auditor Comment	Recommendation	Reviewer Override
Evaluation Medical Review	Contains Medical History	3 / 3	PASS	History names multiple specific, clinically relevant items. Documented: "Prior Medical History: Inclusive but not limited to DM , HTN , Anemia , Back Surgery , L Knee Surgery".	No action needed.	—
Evaluation Medical Review	Contains Medications List	3 / 3	PASS	Specific medication(s) relevant to treatment are named: "Medications Impacting Condition/Treatment: Per consult with medical staff, patient is currently on prescribed medications for HYDROcodone-Acetaminophen Oral Tablet 5-325 MG (Hydrocodone-Acetaminophen) with possible side effects including dizziness and nausea which may affect patient's ability to process / think through	No action needed.	—

Section	Criterion	Score	Status	Auditor Comment	Recommendation	Reviewer Override
				situations safely and logically.".		
Evaluation Medical Review	Evaluation Completed Timely	3 / 3	PASS	Evaluation DOS 5/13/2026; evaluating therapist completion/signature 5/13/2026 12:01:49 PM EDT — elapsed: same day. (Physician certification signature date, if present, is tracked separately and was not used for this calculation.)	No action needed.	—
Evaluation	Health Status / Reason for Referral	3 / 3	PASS	A specific change (Reason for Referral / Current Illness: Patient is an 89 y/o Male with a Pmhx of DM , HTN , Anemia , Back Surgery , L Knee Surgery recent jos...) is documented and reasonably corresponds to the diagnosis on file (Cellulitis of left lower limb / Difficulty in walking, not elsewhere classified).	No action needed.	—
Evaluation	Objective Tests / Standardized Test	3 / 3	PASS	A completed, discipline-appropriate standardized measure was administered: "Test/Sit Balance Sitting Balance Scale" = 37/44.	No action needed.	—
Evaluation	Therapist Assessment	2 / 3	PARTIAL	A clear PLOF-to-CLOF functional change is documented across 13 functional areas (e.g., Roll left and right: Independent → Setup or clean-up assistance; Sit to lying: Independent → Supervision or touching assistance), supporting skilled intervention. Pain is noted but its correlation to goals/diagnosis is not clearly stated. Cross-reference against nursing PCC documentation is flagged separately as	Document a clear, comprehensive comparison between prior level of function and current level of function.	—

Section	Criterion	Score	Status	Auditor Comment	Recommendation	Reviewer Override
				external validation and does not affect this score.		
Evaluation	Orders in PCC	— / 3	UNABLE TO VALIDATE	The uploaded therapy documentation does not contain PCC order evidence. The physician's Plan of Treatment certification within this document is not, by itself, treated as proof of a matching PCC order.	Verify discipline, therapy order, frequency, duration, and effective date directly in PCC.	—
Evaluation	Medical Necessity	3 / 3	PASS	Documentation connects the diagnosis/functional decline (13 functional area(s) documented as declined) to the need for skilled intervention and identifies risk without treatment: "Based upon examination of patient's body regions, systems and structures, patient presents with balance deficits, decreased safety awareness..."	No action needed.	—
Evaluation	Assessment Summary	4 / 4	PASS	Assessment summary synthesizes 4 of 4 expected elements (Clinical impression / reason for skilled services; Risk factors / risk without treatment; Barriers/complexities; Change in functional levels).	No action needed.	—
Plan of Care/Treatment	LTG Justification	1 / 3	PARTIAL	1 of 8 long-term goal(s) do not clearly target an outcome above the documented CLOF/baseline: LTG#3.0 (target "Independence" vs. baseline "Independent").	Confirm each long-term goal's target explicitly exceeds the documented baseline/CLOF and ties to the diagnosis/CPT codes.	—
Plan of Care/Treatment	STG Appropriate and within LTG	1 / 3	PARTIAL	3 of 9 short-term goal(s) could not be linked to any long-term goal: STG#1.0, STG#1.0, STG#3.0.	Ensure every short-term goal is measurable and explicitly supports a documented long-term goal.	—

Corrective Actions

- **Therapist Assessment:** Document a clear, comprehensive comparison between prior level of function and current level of function.
- **LTG Justification:** Confirm each long-term goal's target explicitly exceeds the documented baseline/CLOF and ties to the diagnosis/CPT codes.
- **STG Appropriate and within LTG:** Ensure every short-term goal is measurable and explicitly supports a documented long-term goal.

What Was Done Well

- Clear functional decline is documented across multiple areas (Roll left and right, Sit to lying, Lying to sitting on side of bed), directly supporting the need for skilled intervention.
- Relevant, specific medical history is documented rather than deferring to the chart.
- Reason for skilled services is patient-specific rather than generic boilerplate.
- Short-term goals are appropriate and clearly nest within a long-term goal.

Additional Documentation Findings

- **PLOF/CLOF gap:** 5 functional area(s) documented at evaluation have no matching prior-level-of-function entry: Assistive Device During Transfers, Gait Distance, Assistive Device, Resident uses a wheelchair and/or scooter?, Type of wheelchair or scooter used?.

External Validation Needed

- **Therapist Assessment:** The Key expects the therapist's assessment to be cross-referenced against nursing documentation in PCC, which is not present in the uploaded record.
- **Orders in PCC:** The uploaded therapy documentation does not contain PCC order evidence (a physician's Plan of Treatment certification within the eval document is not, by itself, proof of a PCC order).

AI-assisted clinical documentation review. Findings are intended to support, not replace, qualified clinical review.